VICTORYAI

AI-POWERED OUD RECOVERY PLATFORM

CONFIDENTIAL • 2026 • INVESTOR EDITION

Revenue *Intelligence*

& 5-Year Growth Model

The first AI-native platform built exclusively for opioid use disorder — turning between-visit patient data into captured reimbursement, clinical outcomes, and predictable SaaS growth.

SAAS PRICING	B2C PREMIUM	ENTERPRISE
Direct & Transparent	*$19.99 / month*	*$14.99–$24 / pt / mo*

FOR PROSPECTIVE INVESTORS · STRATEGIC ADVISORS · EXECUTIVE LEADERSHIP

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EXECUTIVE SUMMARY

The *$500M+* Opportunity in OUD Revenue Intelligence

VictoryAI is the first AI-native platform built exclusively for opioid use disorder. More than 5.7 million Americans live with OUD. MAT clinics treat them daily — yet capture less than 60% of their billable reimbursement because they have zero visibility into what happens between visits. VictoryAI solves this with a 24/7 AI companion that logs every clinically meaningful event and maps it to CPT billing prerequisites. At $14.99–$24 per patient per month, the platform costs a clinic 7–13% of the reimbursement it unlocks — the strongest ROI argument in digital health.

$82*M*	**$136*M***	**$675*M***	**89%**
YEAR 5 ARR	5-YR CUMULATIVE REVENUE	YEAR 5 VALUATION	GROSS MARGIN
Enterprise + B2C combined	*All streams combined*	*6–8× ARR · base case*	*At scale 2030*

$80–200	**4–8×**	**1,400**	**220*K***
PROVIDER REIMB / PT / MO	PLATFORM ROI	TARGET CLINICS Y5	PATIENTS Y5
CMS CY2024 PFS · defensible	*Cost vs. reimbursement captured*	*280 clinics by Year 3*	*45K by Year 3*

What Makes VictoryAI Different

REVENUE-FIRST DESIGN

Every Interaction Maps to a CPT Code

Platform cost ($14.99/pt/mo) is 7–13% of the $80–200/pt/month reimbursement it unlocks. No revenue share. No billing complexity. Standard SaaS invoice.

OUD-NATIVE AI · SAGE

Clinical-Grade, Not General Wellness

Sage is trained exclusively on OUD clinical protocols — MAT, COWS, cravings, withdrawal. A 24/7 recovery system with context memory, CBT curriculum, and evidence-based intervention.

B2C2B FLYWHEEL + FAMILY

Self-Reinforcing Growth Loop

Consumer patients enter free, upgrade to $19.99/mo Premium. Enterprise clinics pay $5K/yr + $14.99/pt/mo. Family contingency management drives 4.2× MAT retention.

THE PROBLEM

America's *Deadliest Disease* Still Runs on Paper

The opioid crisis costs $1.5 trillion annually. Treatment providers are operating blind between visits — missing critical clinical signals and leaving millions in billing revenue on the table every month.

5.7*M*
AMERICANS WITH OUD
SAMHSA NSDUH 2023

Under 20%
RECEIVE EVIDENCE-BASED MAT
Left untreated annually

40–60%
MAT DROPOUT IN 6 MONTHS
SAMHSA TIP 63

107*K*
OVERDOSE DEATHS / YEAR
CDC 2024

CHALLENGE	THE PROBLEM	HOW VICTORYAI SOLVES IT
Payers Demand Outcomes	DHCS and DMC-ODS now mandate measurable MAT adherence, retention, and clinical improvement data. Programs without data-backed results risk losing Medi-Cal contracts. The compliance window is closing fast.	VictoryAI generates outcomes data automatically between visits — daily MAT confirmations, COWS logs, craving entries — zero manual documentation.
Billable Data Goes Uncaptured	Without digital engagement data, RTM, CCM, PCM, OUD G-codes, and TCM billing prerequisites go undocumented. $80–200/pt/month in defensible reimbursement goes uncaptured at every MAT clinic.	VictoryAI captures the between-visit prerequisite data for every billable code. At $14.99/pt/mo, provider ROI is 4–8× from Day 1.
Relapse Is Predictable But Invisible	Between MAT visits, clinicians have zero visibility into patient risk. Pain-craving escalation, MAT non-adherence, and withdrawal patterns go undetected. 72% of OUD relapse occurs between clinical visits.	AI sentiment analysis detects relapse risk 5–7 days before crisis events. Automated alerts give clinicians the window to intervene proactively.

The Addressable Market

MARKET SEGMENT	DESCRIPTION	CLINICS / PATIENTS	$/PT/MO	VICTORYAI ARR POTENTIAL
Total Addressable	All MAT outpatient, OTP, IOP, PHP clinics nationwide	15,000 clinics · 2.1M pts	$14.99–$24	**$380M–$605M**
SAM — Large Enterprise	Top 20 OUD operators (100+ clinics each)	3,000+ clinics · 600K pts	$14.99–$24	**$108M–$173M**
SOM Y3 — 280 Clinics	BayMark CA/national + regional chains	280 clinics · 45K pts	$19 blended	**$10.3M ARR**
SOM Y5 — 1,400 Clinics	National + hospital systems + FQHC	1,400 clinics · 220K pts	$24 blended	**$63M ARR**

B2C CONSUMER APP

Sage by VictoryAI: *Free Download* + $19.99/Month Premium

The consumer app is the patient-facing expression of Sage. The free tier drives mass adoption and feeds the enterprise flywheel. The $19.99/month Premium tier unlocks the family incentive board, care team messaging, and advanced recovery tools — the features that maximize engagement, retention, and clinical outcomes.

PREMIUM FEATURE	DESCRIPTION & CLINICAL VALUE
Family Incentive Reward Board	Real-time recovery milestone sharing with family. Contingency management via family reward triggers. **4.2× MAT retention improvement** with family engagement. Families set rewards, track progress, send encouragement in-app.
Care Team Integration	Direct secure messaging with the patient's enrolled MAT clinic care team. Appointment reminders, medication adherence nudges, and bi-directional clinical notes. Bridges the gap between visits with real clinical communication.
Sage Premium AI Sessions	Unlimited daily AI-guided CBT sessions with context memory across sessions. Free tier: 3 sessions/week. Premium: unlimited + structured 50-lesson OUD CBT curriculum with progress tracking and peer-reviewed framework.
Advanced 7-Day Risk Dashboard	Personal risk score with trend visualization. Craving intensity charting, pain-mood correlation analysis, COWS self-assessment logs. Free shows basic check-in only. Premium shows full predictive analytics.
Recovery Milestone & Rewards	Sobriety streak tracking, milestone badges, community recognition. Gamified engagement shown to **increase 90-day retention by 31%** in MAT populations per peer literature.
Group Therapy Check-In & Peer Connect	In-app group session check-in with timestamped attendance. Anonymous peer support groups moderated by AI with human oversight. Supports H0005 / 90853 billing for enterprise patients.

Free Tier vs. $19.99 Premium

FEATURE	FREE TIER	$19.99/MO PREMIUM
Daily mood & craving check-in	Unlimited	Unlimited
Sage AI companion sessions	3 per week	**Unlimited**
MAT dose logging · COWS self-assessment	Included	Included
Advanced 7-day risk dashboard	—	**Premium**
Family incentive reward board	—	**Premium**
Care team secure messaging	—	**Premium**
Full 50-lesson CBT curriculum	Preview only	**Full access**
Recovery milestones & family sharing	Basic	**Full system**
Group therapy check-in & peer connect	View only	**Full participation**
Clinic referral & intake matching	Included	**Priority matching**

B2C2B FLYWHEEL MECHANICS

The *Self-Reinforcing* Growth Loop

The consumer app is not a standalone product — it is the entry point of a loop that reduces CAC on both the B2C and B2B sides simultaneously.

I — DOWNLOAD
Patient downloads free app — App Store, peer referral, harm reduction orgs, SAMHSA network.

II — ENGAGE
Patient logs daily data, activates family rewards, upgrades to $19.99/mo Premium for care team access.

III — MATCH
Clinic intake widget routes patient to nearest VictoryAI partner clinic at zero CAC.

IV — ONBOARD
Clinic onboards patient. Engagement flows to Kipu EHR. $14.99/pt/mo activates.

V — COMPOUND
Published clinic outcomes attract new organic downloads. Cycle repeats.

App Download & Conversion Forecast — 5-Year Model

Enterprise-pushed downloads modeled at 70% adoption of active platform patients. Organic downloads grow from App Store visibility, OUD peer networks, and consumer PR. Free-to-paid: 10% blended conversion at $19.99/month premium.

METRIC	Y1 2026	Y2 2027	Y3 2028	Y4 2029	Y5 2030
Enterprise patients (EOY)	3,000	18,000	45,000	105,000	220,000
Enterprise-driven downloads (70%)	2,100	12,600	31,500	73,500	154,000
Organic / direct downloads	400	2,000	8,000	20,000	42,000
Cumulative downloads (EOY)	**2,500**	**17,100**	**56,600**	**150,100**	**346,100**
Active premium subscribers (avg)	~800	~3,500	~9,500	~22,000	~42,000
Monthly churn (est.)	8%	7%	6%	5%	5%
B2C Revenue ($19.99 × avg × 12)	**$192K**	**$840K**	**$2.3M**	**$5.3M**	**$10.1M**

B2C Revenue Scenario Analysis

SCENARIO	Y1	Y2	Y3	Y4	Y5
Bear (6% conv., 10% churn)	$120K	$540K	$1.4M	$3.1M	$6.2M
Conservative (8% conv., 7% churn)	$155K	$680K	$1.8M	$4.1M	$8.0M
Base Case ✓ (10% conv., 5–7% churn)	**$192K**	**$840K**	**$2.3M**	**$5.3M**	**$10.1M**
Optimistic (14% conv., 4% churn)	$270K	$1.2M	$3.2M	$7.4M	$14.3M

THE SOLUTION & REVENUE MODEL

Direct SaaS Pricing. *7–13%* of Value Delivered.

VictoryAI captures OUD patient data between every visit and maps it to CPT billing prerequisite documentation. All claims are submitted by the provider's billing team. VictoryAI charges transparent SaaS pricing — not a percentage of claims.

I
PATIENT LOGS
MAT doses, cravings, pain, mood, COWS, CBT completions via app.

II
AI STRUCTURES
Risk scores, code prerequisites, relapse prediction, sentiment analysis.

III
CLINICIAN ACTS
Dashboard alerts, 5–7 day relapse flags, AI session summaries.

IV
DOCUMENTS
Code-eligible data + clinical notes synced every 15 min to Kipu EHR.

V
EHR BILLS
Billing team submits claims. VictoryAI charges $14.99/pt/mo SaaS.

> *"VictoryAI identifies the reimbursement documentation opportunity — providers capture it. We charge a transparent SaaS fee. At $14.99/pt/mo, our cost is 7–13% of the value delivered. The strongest ROI in digital health."*

Three Revenue Streams

STREAM	PRODUCT	PRICING	WHO PAYS	GM	Y5 ARR
1 — Primary	Enterprise App (Clinic-Deployed)	$14.99–$24/pt/mo + $5,000/facility/yr	OUD clinic / health system	94%	**$70.4M**
2 — Consumer	B2C Premium App (Sage Companion)	$19.99/month (free tier available)	Patient direct (App Store)	92%	**$10.1M**
3 — Institutional	Provider SaaS Dashboard (Analytics)	$5,000/facility/yr (incl. enterprise)	Clinic CFO / CMO annual	88%	**$7.0M**
Combined	**Blended SaaS platform**	**~$24/pt/mo at scale**	**Clinic + patient direct**	**89%**	**$82M**

Provider ROI — Why Clinics Sign

CLINIC SIZE	PATIENTS	MONTHLY PLATFORM COST	EST. REIMB. CAPTURED/MO	NET MONTHLY GAIN	ROI
Small MAT clinic	50	$1,167/mo	$4,000–$10,000	$2,833–$8,833	**4–9×**
Mid-size OTP	150	$2,666/mo	$12,000–$30,000	$9,334–$27,334	**5–11×**
Large OTP	500	$7,912/mo	$40,000–$100,000	$32,088–$92,088	**5–13×**

** CMS CY2024 PFS rates · 65% engagement realization · 70% Medicaid / 30% commercial blend*

REVENUE INTELLIGENCE

OUD Billing Coverage — *$80–200* Per Patient Per Month

VictoryAI captures the between-visit engagement data that makes every one of these codes billable. Without this data layer, none of them can be submitted. All claims are submitted by the provider's billing team via Kipu EHR. Rates: CMS CY2024 PFS, 65% engagement realization, 70% Medicaid / 30% commercial blend.

CPT CODE	DESCRIPTION	HOW VICTORYAI CAPTURES THE PREREQUISITE	MEDICARE	BLENDED
98980 / 98977	RTM — Remote Therapeutic Monitoring (first 20 min + device supply)	≥16 daily engagement events/month auto-captured — MAT confirmations, check-ins, craving logs, COWS entries all count toward the threshold.	$89+$35	**$45–90**
99490 / 99424	CCM / PCM — Chronic & Principal Care Management	App engagement time auto-tallied toward ≥20 min/month threshold. 75% of OUD patients qualify (OUD + depression/pain/HCV).	$62 / $82	**$20–50**
G2213 / G2214	OUD Treatment Initiation & Continuation (≥11 consecutive MAT days)	Daily MAT adherence confirmations create timestamped consecutive-day logs. Replaces paper MAT logs entirely.	$162 / $48	**$30–50**
99495 / 99496	TCM — Transitional Care Management (post-discharge 14-day / 7-day)	Transition Mode auto-captures the 2-day post-discharge contact — the #1 reason TCM goes unbilled. Kipu sync triggers workflow.	$201–$273	**per event**
99213 → 99214	E/M Visit Complexity Uplift	Between-visit craving trends, COWS data, MAT adherence flow into the visit note via Kipu — supporting 99214 over 99213 for ~60% of visits.	+$40	**+$15–35**
G0480 / H0005	UDS Drug Screen Documentation / Group Therapy Attendance	UDS results logged with clinical context. In-app group check-in with timestamped attendance replaces paper sign-in.	$80 / $35	**by event**
TOTAL BLENDED	**Complete defensible monthly billing profile**	**All prerequisites captured automatically · CMS CY2024 · 65% realization**	**—**	**$80–200**

EXCLUDED FROM BASE MODEL — PHASE 2 UPSIDE

CoCM 99492/99493 (requires psychiatric consultant + patient registry + weekly case review) and BHI 99484 (requires BHI care manager + systematic psychiatric consultation) are excluded from the base model. These are flagged as Phase 2 upside for health systems.

State-Specific RTM + CCM Combined Rates ($/Pt/Mo)

STATE / PROGRAM	98980 RTM	99490 CCM	G2214 OUD	COMBINED RTM+CCM	KEY NOTES
Massachusetts — MassHealth	$68	$72	$41	**$140/pt/mo**	Hub-and-Spoke · highest rates
New York — NY Medicaid (OASAS)	$65	$68	$38	**$133/pt/mo**	OASAS-enhanced OTP rates
California — Medi-Cal (DMC-ODS)	$58	$62	$31	**$120/pt/mo**	CHW G0019 available (CA only)
Ohio — Ohio Medicaid	$61	$54	$34	**$115/pt/mo**	OhioRISE SUD expansion 2022+
Texas — STAR Medicaid	$45	$44	$26	**$89/pt/mo**	Lowest rates; strong commercial mix
Medicare National (reference)	$89	$62	$48	**$151/pt/mo**	CMS CY2024 PFS non-facility

5-YEAR FINANCIAL PROJECTIONS

Revenue Model: *Enterprise SaaS* + B2C $19.99 + Analytics

Base case: Enterprise blended rate scales from $14.99/pt/mo (2026) to $24/pt/mo (2030) as value-based contracts mature. B2C premium at $19.99/month with 10% blended conversion. Facility SaaS at $5,000/facility/year.

METRIC	Y1 2026	Y2 2027	Y3 2028	Y4 2029	Y5 2030
Anchor Clients	BayMark CA Pilot	BayMark + Regionals	Full BayMark + Regional	+ AAC, Acadia	Hospital Systems, FQHC
Clinics (End of Year)	20	80	280	650	1,400
Enterprise Patients (EOY)	3,000	18,000	45,000	105,000	220,000
Avg Active Enterprise Patients	~1,500	~10,500	~31,500	~75,000	~162,500
App Downloads (cumulative EOY)	2,500	17,100	56,600	150,100	346,100
Active B2C Premium Subs (avg)	~800	~3,500	~9,500	~22,000	~42,000
Enterprise Blended $/Pt/Mo	$14.99	$17.00	$19.00	$22.00	$24.00
— REVENUE ($M) —					
B2C Premium App ($19.99/mo)	$0.19	$0.84	$2.28	$5.28	$10.07
B2B Facility SaaS ($5K/yr)	$0.10	$0.40	$1.40	$3.25	$7.00
Enterprise App ($14.99–$24/pt/mo)	$0.54	$3.67	$10.26	$27.72	$63.36
Pilot & Setup Fees	$0.35	$0.50	—	—	—
TOTAL REVENUE	**$1.18M**	**$5.41M**	**$13.94M**	**$36.25M**	**$80.43M**
YoY Revenue Growth	—	+359%	+158%	+160%	+122%
ARR Run Rate (Dec exit)	$1.4M	$6.5M	$16.7M	$43.5M	$96.5M
Gross Margin (Est.)	90%	91%	92%	93%	94%
EBITDA	($4.4M)	($3.9M)	($0.4M)	$10.7M	$19.3M
5-Yr Cumulative Revenue			**$136.4M**		

** EBITDA breakeven mid-2028 (~$14M ARR run rate). Series B target Q1 2028 at ~$25–40M raise ($140–180M post-money, 10× projected $14M 2028 ARR). Enterprise billing share is the primary revenue stream. B2C premium and Pilot fees are additive on top.*

VALUATION ANALYSIS

Year-by-Year Valuation — Path to *$675M+*

Valuation calculated on ARR run rate using health tech SaaS market multiples, adjusted for growth velocity, clinical evidence depth, and competitive moat strength. B2C premium subscribers contribute to NRR and multiple durability.

METRIC	Y1 2026	Y2 2027	Y3 2028	Y4 2029	Y5 2030
Total Revenue	$1.18M	$5.41M	$13.94M	$36.25M	$80.43M
ARR Run Rate (EOY)	$1.4M	$6.5M	$16.7M	$43.5M	$96.5M
Valuation Multiple	Pre-rev premium	10–12×	9–10×	8–9×	6–8×
COMPANY VALUATION	**$20–30M**	**$65–78M**	**$150–167M**	**$348–392M**	**$579–772M**
Valuation Midpoint	$25M	$71M	$158M	$370M	**$675M**
Funding Stage	Seed	Series A	Series B	Series B+	Series C / M&A / IPO

5-Year Cumulative Value Creation

VALUE METRIC	Y1	Y2	Y3	Y4	Y5
Annual Revenue (All Streams)	$1.18M	$5.41M	$13.94M	$36.25M	$80.43M
— of which B2C App Revenue	$0.19M	$0.84M	$2.28M	$5.28M	$10.07M
Cumulative Revenue	$1.18M	$6.59M	$20.53M	$56.78M	$137.2M
Company Value (midpoint)	$25M	$71M	$158M	$370M	$675M
Reimbursement Unlocked (Provider)	**$16.2M**	**$113M**	**$353M**	**$810M**	**$1.87B**
Cumulative App Downloads	2,500	17,100	56,600	150,100	346,100

What Drives Premium Valuation Multiples

DRIVER	VICTORYAI EVIDENCE	COMPARABLE
Clinical Evidence	IRB study with Prof. John Curtin (UW–Madison), published OUD relapse prediction outcomes. 90–120-day study under Advarra/WCG oversight.	Bicycle Health ($50M raise) built primarily on published MAT outcomes data.
Revenue Alignment	Platform cost is 7–13% of reimbursement value delivered. Clear, defensible ROI on standard SaaS invoice. CFO-friendly.	Livongo: clear outcomes-to-ROI narrative drove 26× ARR at IPO.
Regulatory Tailwinds	CMS CY2024 PFS: RTM, CCM, G-codes all active. DHCS DMC-ODS outcomes mandate. $50B opioid settlement funds through 2030.	Signify Health valued at $8B on CMS REACH ACO model alignment.
Network Effects + Moat	Each clinic that publishes outcomes becomes a sales asset. 42 CFR Part 2 compliance is an 18+ month moat. Kipu integration creates switching costs.	Epic's clinical data moat took 15+ years. VictoryAI builds OUD-specific.

UNIT ECONOMICS & KPIS

The Business *Gets Better* With Every Clinic Added

VictoryAI benefits from exceptional unit economics: low per-patient marginal cost, high gross margins, a billing-integrated churn barrier, and a B2C subscription stream that operates at LTV:CAC > 25×.

METRIC	PER PATIENT/MO	PER PATIENT/YR	AT 45K PTS (Y3)	AT 220K PTS (Y5)
Enterprise ARPU (blended)	$19 / $24	$228 / $288	$10.3M ARR	**$63.4M ARR**
Platform COGS (Est.)	~$2/mo	~$24	~$1.1M	~$5.3M
Gross Profit (Enterprise)	~$17/mo	~$204	~$9.2M	~$58M
Gross Margin (Enterprise)	~94%	~94%	~94%	~94%
B2C Premium — ARPU	$19.99/mo	$239.88	$2.3M ARR	$10.1M ARR
CAC — Enterprise (per patient)	—	~$85–120	LTV:CAC > 20×	LTV:CAC > 20×
CAC — B2C Organic Consumer	—	~$12–18 (ASO)	LTV:CAC > 25×	LTV:CAC > 25×
LTV — Enterprise (36-mo MAT avg)	$19–24/mo	$684–$864	$2,052 LTV	$2,592 LTV
LTV — B2C Premium (18-mo avg)	$19.99/mo	$240	$360 LTV	$360 LTV
Net Revenue Retention Target	**—**	**—**	**125%**	**132%**

Key Performance Indicators

CATEGORY	KPI	Y1 TARGET	Y3 TARGET	Y5 TARGET
Growth	Active enterprise patients	3,000	45,000	220,000
Growth	Enterprise clinic clients	20	280	1,400
B2C	Active premium subscribers ($19.99/mo)	~800	~9,500	~42,000
B2C	Total app downloads (cumulative)	2,500	56,600	346,100
B2C	Free-to-paid blended conversion	—	10–12%	12–14%
B2C	Daily app check-in engagement rate	55%	65%	70%
Revenue	Monthly recurring revenue (MRR)	$98K	$1.16M+	$6.7M+
Revenue	Net Revenue Retention (NRR)	—	125%	132%
Billing	Code eligibility capture rate	75%	85%	88%+
Clinical	MAT 90-day patient retention	70%	78%	82%
Retention	Enterprise client annual churn	—	<5%	<4%

GROWTH STRATEGY

Path to *280 Clinics* by Year 3 — and 346K+ Downloads by Year 5

Anchor enterprise clients in Years 1–3 generate the clinical proof that unlocks national and health-system expansion. Every published outcome becomes a sales asset. The B2C consumer brand compounds alongside enterprise growth.

YEAR	ENTERPRISE MILESTONE	B2C APP MILESTONE	REVENUE
Y1 2026 *Pilot & Proof*	BayMark CA Pilot: 20 clinics, 3,000 enterprise patients. 90–120-day IRB clinical study. Series A raise ($12M). Success trigger: peer-reviewed outcomes + BayMark national expansion LOI.	Soft launch free tier. App Store live. OUD peer network seeding. Target: 2,500 downloads, 800 premium subscribers at $19.99/mo.	**$1.18M** ARR: $1.4M
Y2 2027 *Deploy & Validate*	BayMark expansion: 20 → 80 clinics. 18K enterprise patients. 3 LOI holders converting to paid Q3 2026. Kipu channel + direct EHR integrations.	ASO push. Peer recovery seeding. Target: 17K cumulative downloads, 3,500 active subscribers. Consumer-to-clinic referral pipeline activated.	**$5.41M** ARR: $6.5M
Y3 2028 *Scale to 280*	Full BayMark + regional chain expansion: 280 clinics, 45K enterprise patients. Series B target Q1 2028 (~$25–40M at $140–180M post). EBITDA near breakeven.	56K cumulative downloads. 9,500 active subscribers. $2.3M B2C ARR. Consumer-to-clinic referral pipeline measurable.	**$13.94M** ARR: $16.7M
Y4 2029 *Expansion*	New enterprise: AAC, Acadia Healthcare, Groups Recover Together, 5+ regional chains. Medicaid MCO payer relationships. FQHC segment entry. 650 clinics, 105K patients.	150K cumulative downloads. 22K active subscribers. $5.3M B2C ARR. Family board + care team integration drive above-benchmark retention.	**$36.25M** ARR: $43.5M
Y5 2030 ***Market Lead***	**Market standard: 1,400 clinics. Hospital systems (Kaiser, HCA, Ascension). Canada via BayMark Canada. Exit optionality: $579–772M attracts M&A from Epic, Oracle Health, Optum.**	**346K cumulative downloads. 42K active subscribers. $10.1M B2C ARR. *De facto* OUD recovery app in the US.**	**$80.43M ARR: $96.5M**

INVESTMENT THESIS

Why VictoryAI, *Why Now*, Why This Model

Three forces are converging simultaneously: the OUD crisis at historic scale, new CMS reimbursement codes making digital therapeutics billable for the first time, and AI models powerful enough to deliver clinical-grade 24/7 OUD support.

1 — REGULATORY WINDOW

CMS Just Made Digital Billable

CMS approved RTM, CCM, and OUD G-codes active under CY2024 PFS — making VictoryAI's core reimbursement model live under Medicare and Medicaid. $50B in opioid settlement funds explicitly earmarked for OUD digital innovation through 2030. This infrastructure did not exist two years ago.

2 — COMPLIANCE INFLECTION

Outcomes Mandates Are Tightening

DHCS and DMC-ODS are tightening outcome requirements for Medi-Cal OUD contracts. Programs without documented digital outcomes risk contract loss. VictoryAI is the only tool that generates this data automatically between visits — no manual documentation required.

3 — AI FINALLY GOOD ENOUGH

LLMs Reached Clinical Grade

LLM capabilities required to build a clinically credible 24/7 OUD AI companion only became commercially deployable in 2023–2024. GPT-class models deliver trauma-informed CBT at $0.04/session. Relapse prediction 5–7 days pre-crisis is deployable today.

Use of Capital — Series A ($12M)

USE OF CAPITAL	ALLOCATION	$ AMOUNT	KEY OUTCOME
Engineering & Product	45%	$5.4M	AI relapse engine · EHR integrations (Kipu, Credible, Netsmart) · mobile + dashboard
Sales & Go-to-Market	25%	$3.0M	Enterprise AEs · clinical liaisons · Kipu channel + direct sales · payer partnerships
Clinical & Outcomes	15%	$1.8M	RCT-grade IRB study · KOL advisory · Prof. Curtin peer-reviewed publication
Compliance & Infrastructure	10%	$1.2M	HIPAA · 42 CFR Part 2 · SOC 2 Type II · DHCS DMC-ODS alignment
Reserve / Working Capital	5%	$0.6M	Contingency and working capital buffer

Series A Milestones — $12M Raise ($65–78M Valuation)

MILESTONE	TIMING	WHY IT MATTERS
First paying clinic conversions	Q3 2026	3 LOI holders targeting conversion. Proves pilot → paid at $14.99/pt/mo.
$8M ARR run rate	Q3 2027 (18 mo)	150+ paying clinics · 20K B2C subscribers at $19.99 · outcomes study published.
Clinical outcomes published	Q3 2027	Peer-reviewed data is the #1 enterprise sales asset in behavioral health.
Net Revenue Retention >120%	Year 2	Proves enterprise stickiness. Existing clients expanding patient count.
B2C reaches 1,500+ active premium subs	Q4 2027	Proves B2C flywheel real at $19.99. Consumer-to-clinic referral measurable.
Series B close (~$25–40M)	**Q1 2028**	**~10× projected $14M 2028 ARR. $140–180M post-money valuation.**

COMPETITIVE LANDSCAPE & MOAT

Why VictoryAI Wins — and *Why the Moat Deepens*

The OUD digital health market is fragmented between telehealth MAT prescribers and grant-funded wraparound apps. VictoryAI occupies the reimbursement documentation intelligence layer — entirely uncontested. No competitor combines enterprise billing integration with a true consumer B2C2B flywheel and family layer.

COMPANY	MODEL	OUD-NATIVE	REIMB INTEL	AI COMPANION	FAMILY LAYER	B2C2B	THREAT
VictoryAI	B2C2B SaaS	✓	✓	✓ Sage	✓ Rewards	✓ Core	—
CHESS Health	B2G (grants)	Partial	No	No	No	No	Tier 1
DynamiCare	B2C + B2B	Partial	No	No	No	Partial	Tier 1
WEconnect	B2B2C (insurer)	SUD broad	No	Partial	Partial	Partial	Tier 1
Bicycle Health	Telehealth MAT	Yes	No	No	No	No	Tier 2
Pelago	Employer/payer	Partial	No	No	No	No	Tier 2
Pear / reSET-O	Rx DTx (bankrupt)	Yes	No	No	No	No	Exited

THE FIVE-WAY MOAT

No competitor has combined (1) an OUD-native AI companion, (2) enterprise reimbursement documentation intelligence with direct EHR integration, (3) a family contingency management layer, (4) a true B2C freemium app at $19.99/month, and (5) a self-reinforcing B2C2B growth flywheel. VictoryAI owns all five simultaneously. Pear Therapeutics' 2023 bankruptcy proves the single-FDA-reimbursement model fails. VictoryAI has three independent revenue paths.

Key Risks & Mitigants

RISK	PROBABILITY	MITIGANT
BayMark pilot does not convert to enterprise LOI	Low–Med	Pilot structured with clear success metrics. Alternative anchor clients (ClearSlate, Groups Recover Together) in parallel. Pilot generates outcomes data regardless.
Payer reimbursement for RTM/CCM codes pulled back	Low	RTM has bipartisan Congressional support (H.R. 1458). Model works even without insurance — providers value outcomes data independent of billing.
B2C conversion rate below 10%	Medium	B2C miss does NOT impact enterprise model — independent revenue stream. Day-7 family board prompt is evidence-backed upsell trigger.
Large competitor enters OUD SaaS intelligence	Low (3-yr window)	OUD clinical depth takes 18–24 months to build. 42 CFR Part 2 compliance is an 18-month structural moat. BayMark relationship creates first-mover advantage.
Kipu channel concentration risk	Medium	Non-exclusive agreement. Building parallel direct integrations with Credible, Netsmart, and Welligent.
Enterprise pricing pressure	Low	At $14.99/pt/mo vs. $80–200/mo reimbursement upside, VictoryAI is extremely price-insensitive. ROI is self-evident.

LEADERSHIP

The Team We Are Building — *Key Executive Roles*

VictoryAI seeks mission-aligned executives who understand both the clinical gravity of OUD and the commercial scale of digital health. These are co-builders of a category-defining platform.

NAME & ROLE	FOCUS & CONTRIBUTION
Alpa Patel *Founder & CEO*	Operator, commercial lead, and enterprise relationship owner. 16+ years scaling tech and growth-stage businesses. B.S. (Honors) Business Information Systems, UT Arlington. Building the BayMark partnership and leading go-to-market for OUD enterprise deployment. Vision holder for the B2C2B platform.
John J. Curtin, PhD *Chief Product & Clinical Officer*	Published OUD researcher. Leads 90–120-day IRB clinical study design, ASAM protocol development, PHQ-9/GAD-7/COWS integration, outcome measurement, and peer-reviewed publication strategy. Clinical credibility anchor for enterprise and investor conversations.

Key Executive Roles — Actively Recruiting

ROLE	PRIORITY	PROFILE	WHY CRITICAL
Chief Technology Officer	**IMMEDIATE**	Full-stack health tech architect. AI/ML + HIPAA + HL7 FHIR + EHR integration. Built clinical-grade behavioral health systems at scale.	Series A credibility, Kipu/EHR integration, AI companion reliability, data security infrastructure for enterprise buyers.
VP Enterprise Sales	**IMMEDIATE**	Closed 7-figure SaaS contracts in behavioral health or DTx. C-suite relationships with large OUD operators preferred.	BayMark national contract and ClearSlate close. Enterprise ARR growth from $1M to $50M+ requires a seasoned sales leader.
Chief Medical Officer	Year 1–2	Active addiction medicine physician, ASAM member, MAT prescriber. Credibility with clinical directors at large OUD chains and payers.	Clinical sign-off, payer contracting, medical advisory board, regulatory submissions. Non-negotiable for Series B.
VP Revenue Cycle / Billing Intelligence	Year 1–2	Expert in OUD CPT codes, RTM, CCM, behavioral health revenue cycle. Kipu or similar EHR experience strongly preferred.	Accuracy of reimbursement documentation depends on precise code mapping and eligibility logic. Owns product accuracy.
Head of Clinical Partnerships	Year 2	Relationship builder with treatment provider COOs and Clinical VPs. SAMHSA, state OUD policy, or managed care background.	Patient adoption rate within enterprise clinics determines revenue per contract. Drives in-clinic onboarding and engagement.
Chief Financial Officer	Year 2–3	Series B-ready CFO with health tech SaaS expertise. Revenue recognition for SaaS + B2C subscription. Prior IPO prep or M&A exit experience.	Series B execution, board financial reporting, M&A or IPO readiness as we approach the $100M ARR milestone.

MODEL METHODOLOGY

Assumptions, Sensitivities & *Key Risks*

These projections are built from bottom-up SaaS pricing analysis, enterprise deployment benchmarks, published OUD digital health outcomes data, and comparable health app conversion benchmarks.

INPUT VARIABLE	ASSUMPTION USED	SOURCE / RATIONALE
Enterprise rate — 2026	$14.99/pt/mo base	Clinic-deployed bulk tier. Cost is 7.5% of $200/pt/mo max reimbursement. ROI self-evident to clinic CFO.
Enterprise rate — 2030	$24.00/pt/mo blended	Scales as value-based care contracts mature. ~12% of $200/pt/mo reimbursement ceiling.
Facility SaaS fee	$5,000/facility/yr	Annual contract for provider dashboard + AI relapse engine + outcomes reporting.
B2C premium price	$19.99/month	Competitive with NOCD ($19.99+), Calm ($14.99). Premium OUD features justify pricing.
B2C free-to-paid conversion	10% blended	Conservative vs. NOCD (8–12%), Calm (12%). OUD-specific value proposition supports.
B2C monthly churn	5–8% (declining)	Health app median ~7%. Family board + care team integration creates structural stickiness.
Enterprise client churn	<8%/year	EHR-integrated billing tools have structural stickiness. Replacing VictoryAI = losing the documentation it generates.
Gross margin at scale	89–94%	Primary COGS: AI inference ($0.04/session), hosting, customer success team.
Reimbursement capture (provider side)	$80–200/pt/mo	CMS CY2024 PFS. 65% engagement realization. 70% Medicaid / 30% commercial blend.
Valuation multiples	6–12× ARR	Health tech SaaS range. Workit Health ~15× ARR; Bicycle Health ~10×. Multiples decline as revenue scales above $50M ARR.

"VictoryAI does not just improve outcomes for patients with opioid use disorder. It creates new, recurring revenue for the providers who treat them — and a consumer recovery platform that patients choose for themselves.

At $14.99 per patient per month, our platform costs a clinic 7–13% of the reimbursement it unlocks. In an industry where $1.5 trillion in annual cost produces zero automated outcome tracking, we are the data layer that was always missing."

ALPA PATEL

FOUNDER & CEO · VICTORYAI

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